# Multiple Trauma

**TB-06 | Trauma & Burns**

## Quick flow

**01 Major Trauma [EMT / AEMT / Paramedic]**

Scene safety • mechanism • resources • request air medical early when it meaningfully improves time to needed care or provides unavailable capability; never delay ground transport.

**02 XABCDE [EMT / AEMT / Paramedic]**

Exsanguination • airway • breathing • circulation • disability • exposure.

**03 High-Risk Trauma? [EMT / AEMT / Paramedic]**

Physiologic abnormality • high-risk anatomy • special considerations (age, anticoagulants, pregnancy, burns with trauma, suspected abuse) • concerning mechanism.

**04 Treat Immediate Threats [EMT / AEMT / Paramedic]**

Catastrophic hemorrhage control • airway/ventilation • assess chest injury and urgently request ALS for suspected tension pneumothorax • pelvic stabilization only when indicated • warming • reassessment.

**05 Chest Decompression [Paramedic]**

Paramedic-only chest decompression for suspected tension pneumothorax under current standing orders; reassess ventilation and perfusion after intervention.

**06 Resuscitate En Route [AEMT / Paramedic]**

Access • hemorrhage control • warmed isotonic crystalloid titrated to perfusion when blood is unavailable • higher adult TBI SBP target ≥110 mmHg • analgesia • warming • serial reassessment. Follow AM-05; avoid unnecessary large-volume crystalloid.

**07 Trauma TXA Eligibility [Paramedic]**

Paramedic only: after immediate lifesaving interventions, consider IV/IO TXA for traumatic hemorrhage with clinical hemorrhagic shock within 3 hours of injury. Adult dose: 1-2 g IV/IO: give 1 g over 10 minutes or 2 g over 20 minutes; do not exceed 100 mg/min. Verify the stocked concentration and dilution against the current medication-preparation instructions. Do not administer an automatic repeat dose; any additional dose requires a separate authorized order.

**08 Appropriate Destination [EMT / AEMT / Paramedic]**

Apply the trauma field-triage and destination plan: high-risk patients to the highest-level appropriate trauma center within regional constraints; favor pediatric-capable trauma care for children. Use a closer stabilization hospital only for an immediate critical need that cannot safely be managed during longer transport, under the destination plan/Medical Control. Notify early, document the reason, and activate the predefined transfer process. Do not delay ground movement solely while awaiting aircraft.

## Overview

- Use a rapid XABCDE approach, treat immediate threats, and minimize scene time for high-risk trauma.
- Destination selection and early notification are part of treatment.

## Provider-Level Actions

Perform all immediately indicated care within scope without delaying rapid trauma transport.

**EMT**

- Perform rapid XABCDE, control hemorrhage, support airway/ventilation, assess chest injury, oxygenation and perfusion, and urgently request ALS for suspected tension pneumothorax. Stabilize the pelvis only when indicated, stabilize fractures, and prevent hypothermia.

- For life-threatening extremity hemorrhage, apply an approved tourniquet promptly when direct pressure is ineffective or impractical. Tighten until bleeding stops and the distal pulse is absent. If bleeding persists despite proper placement and tightening, add a second tourniquet immediately proximal to the first when feasible. Record time, leave visible, and reassess after movement and throughout transport. For suitable junctional wounds, pack with approved gauze and maintain direct pressure per product instructions. Do not periodically loosen or remove a tourniquet.
- For an open chest wound in a spontaneously breathing patient, consider an approved vented seal and reassess frequently. If deterioration follows sealing, assess/release an obstructing seal and obtain immediate Paramedic treatment for suspected tension pneumothorax. Do not probe the wound. Positive-pressure ventilation requires the chest-injury standing orders and immediate Paramedic/Medical Control guidance, not automatic extrapolation of this seal instruction.
- Package promptly; complete the secondary examination during transport when possible.

**AEMT**

- Perform all EMT care plus IV/IO access, age-appropriate authorized shock resuscitation, and airway support during transport. For adult hemorrhagic shock when blood is unavailable, follow AM-05: titrate warmed isotonic crystalloid to perfusion; avoid unnecessary large-volume crystalloid. If TBI is suspected, prevent hypotension and target adult SBP at least 110 mmHg; do not use permissive hypotension.
- Trend mental status, perfusion, respiratory status, and response to treatment.

**Paramedic**

- Perform all prior care plus advanced airway treatment, chest decompression, analgesia, and protocol-directed hemorrhagic-shock resuscitation.
- Paramedic only: after immediate lifesaving interventions, consider IV/IO TXA for traumatic hemorrhage with clinical hemorrhagic shock no more than 3 hours after injury. Adult dose: 1-2 g IV/IO: give 1 g over 10 minutes or 2 g over 20 minutes; do not exceed 100 mg/min. Verify the stocked concentration and dilution against the current medication-preparation instructions. Do not administer an automatic repeat dose; any additional dose requires a separate authorized order. Do not infer a systemic trauma regimen from topical indications. Record injury time and any TXA dose/time, and notify the receiving team to prevent duplicate dosing.
- Pediatric trauma TXA: local dose reference 15 mg/kg, Paramedic-only. Before administration, confirm pediatric eligibility, maximum dose, administration rate, and preparation with Medical Control within provider scope. Do not extrapolate the adult dose or infusion rate to children.
- Use blood products only within an authorized, supported transfusion program with required training, monitoring, and destination coordination.
- Lead destination, air-medical, and Medical Control decisions without extending scene time.

## Indications

- Patient with suspected multiple trauma requiring prehospital assessment or treatment.

## Warnings / precautions

- Do not delay transport for IV attempts, detailed examination, splinting, or procedures that can be completed en route.
- Prevent hypothermia throughout trauma resuscitation.
- Trauma TXA is Paramedic-only, not an AEMT intervention. Adult dose: 1-2 g IV/IO: give 1 g over 10 minutes or 2 g over 20 minutes; do not exceed 100 mg/min. Verify the stocked concentration and dilution against the current medication-preparation instructions. Do not administer an automatic repeat dose; any additional dose requires a separate authorized order. Pediatric administration requires confirmation of the patient-specific maximum and rate with Medical Control within provider scope. Do not infer a trauma regimen from topical bleeding instructions.
- For pediatric traumatic hemorrhage, obtain Medical Control guidance and use only the authorized pediatric trauma resuscitation order; do not apply medical or septic-shock fluid targets indiscriminately.

## Related pathways / contacts

**AM-05 Hypotension / Shock** - Adult hemorrhagic-shock strategy and suspected-TBI perfusion exception.

**TB-12 Abdominal / Pelvic Trauma** - Concealed hemorrhage, pelvic stabilization, and trauma TXA eligibility, the authorized adult dose range, and outstanding administration parameters.

**TB-08 Spinal Motion** - Open when selective spinal motion restriction is indicated.

**TB-10 Traumatic Arrest** - Open for traumatic pulseless arrest.

### Source and references

Current Tennessee EMS scope of practice, verified individual credentials, and Claiborne County standing orders control provider authorization.

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